

Standard Operating Procedure for ED and Pre-Hospital Referrals to the Stroke Services at Hull University Teaching Hospitals

Background:

HUTH provides the acute stroke services for the Hull and East Yorkshire regions. The first point of contact/referral at HUTH is the Stroke Coordinator (bleep 312). There is also a stroke registrar (SR) on call 7 days a week 9am-5 pm on bleep 010. The SPOC (STROKE physician on call) provides 24/7 support, physically between 9-5pm Monday to Friday and 9-2pm weekends and remotely out with these times.

The referrals can be made from pre-hospital services e.g. Primary care, Ambulance services (YAS, EMAS) and from the Emergency Department. The Neurosciences Care Group understand their limitations with staffing and a dedicated assessment area.

Aim:

The primary aim of this pathway is to ensure that patients who require the acute stroke services are referred to and managed appropriately and in a timely manner by the correct service provider. It is also intended that those patients who are identified with other clinical conditions, but initially seen by the Stroke coordinator, can be referred on and managed appropriately for their alternative diagnoses. The SPOC is available for advice on non-acute or atypical presentations but cannot provide any support for acute medical illnesses or non- stroke neurological pathology. There is an acute neurology service available 24/7.

This SOP will also help to clarify any areas on the uncertainty during the referral process.

Stakeholders:

Acute and Emergency Medicine Care Group
Neurosciences Care Group

Key Points:

1. All suspected stroke presentations to be referred to the stroke coordinator (bleep 312)- from ED and from YAS/EMAS
2. Patients accepted to be reviewed by the stroke team, should be reviewed within 30 mins of referral, wherever feasible.
3. The Stroke coordinator should discuss all referrals with the Stroke Consultant/Middle Grade Clinician/RMO2
4. The Stroke coordinator cannot ask for ED to organise imaging prior to reviewing the patient directly.
5. For any suspected stroke, a review is necessary before imaging is requested.

6. For patients declined for review by the stroke coordinator or felt not likely to be a stroke after review, these will be seen in time order by an ED clinician. A decline to review does not mean a stroke has been ruled out. An early review in the ED is however prudent to ensure that time limited treatments can still be given if needed.
7. If ED medical team have reviewed a patient and a decision is needed regarding appropriate imaging then they should contact the SPOC who can advise on either a stroke medical review or imaging without stroke medical review, depending on the threshold of suspicion conferred by details provided.
8. ED triage service will now use the ROSIER scoring system prior to referring to the stroke team.
9. If expertise on clinical management of a patient already reviewed by ED senior team is needed, then a direct conversation with the stroke consultant (SPOC) is encouraged.
10. ED will directly support the management of any acutely unwell patient suspected of having a stroke
11. Out of hours provision of on-site senior advice/support to the stroke coordinator is from stroke reg/RMO2
12. If there are concerns around the management of this group of patients it can be raised with the SPOC and/or the EPIC of that area

Referrals back to ED:

1. The Stroke coordinator, after review of the patient, can hand the patient back to ED, if they feel that the patient does not have an acute stroke and may have an alternative diagnosis. These patients should however have been discussed with the SPOC before handing back to ED.
2. The stroke coordinator is expected to document that on Lorenzo via "Refer to specialty" and through "void seen". They must also inform the original referrer that this patient is now to be seen in ED.
3. This patient will then be seen in time order by an ED clinician
4. If however, the patient has been reviewed by a SPOC or SR- they must not refer back to ED. They are expected to either admit, discharge or refer on.
5. All plans to be communicated verbally and via Lorenzo.

Specific Neurosciences Conditions:

1. Non-traumatic Intra-cerebral Haemorrhage (ICH): the stroke service will liaise with the neurosurgical team regarding management. But generally, most non-traumatic ICH (apart from Sub Arachnoid Haemorrhage or ICH related with ruptured AVM or DAVF diagnosed on CTA)) cases will be admitted under the stroke service
2. Traumatic ICH: All traumatic ICH (contusions, SDH, EDH, t-SAH etc) will be referred to the Neurosurgical team for further management and should not be referred to stroke. The Major Trauma admitting guidelines can assist with this
3. Intra-Cranial Space Occupying Lesion (ICSOL): If this is found on imaging, then a NS referral should be made via referapatient for MDT follow-up and plan. The patient would be admitted under Acute Medicine or DME (based on age/CFS) if there is a need for hospital admission- i.e. it meets a criteria to admit. The patient

can be discharged if there is no acute reason requiring hospital admission.
Neurosurgery team will organise follow-up.

4. Any incidental finding on imaging will be managed by the stroke service or requesting team (as per Trust guidelines) and referred on.

TIA referrals:

1. 7am – 9pm: referrals made to the stroke coordinator who will discuss with the SPOC
2. After 9pm: ED to manage and make an online referral.
3. No acute imaging required prior to referral.
4. ED to ensure an appropriate history has been documented.

Benefits of the pathway:

- All suspected Stroke patients will get an early review by the stroke team
- The stroke coordinator can hand back non-stroke patients thus helping them to focus on the acute stroke patients.
- Reduction in variability in the choice of imaging by the stroke team, alignment with NOSIP leading to shorter stay in ED
- Reduction in duplication of imaging

Risks:

- Increased workload on the stroke team
- Lack of a stroke assessment unit which could help address the above concerns
- Increased stay in ED till final diagnosis, thus impacting on 4-hour KPI
- Likelihood of patients who are referred back to ED breaching the 4-hour KPI

Monitoring and Forward Plans:

- This pathway will be monitored regarding compliance and any concerns raised
- This pathway has been approved by the Triumvirates from both Care Groups and approved at the ED Clinical Governance meeting.
- Modifications to this pathway will be made by the Clinical Leads/ Clinical Directors from both care groups following mutual agreement and marked as addendums
- The stroke service accepts that they do not currently have adequate staff at both clinician and specialist nurse levels to meet the demands of the service. They also accept that this could lead to delayed care to suitable patients

For review in December 2026

(agreed by stroke services on 28/11/2025)